
Subject: Influenza and Winter Illness Report 2024
Date of report: 04 September 2025

Introduction

This report summarises influenza statistics over the 2024-2025 winter period, including:

- number of resident patients presenting to their GP with flu-like illness
- annual uptake of the seasonal influenza vaccination

Clinicians can use the numbers presenting with 'flu-like' symptoms, alongside cases of influenza confirmed amongst patients to assess the extent of influenza on the Island.

The report also shows to what extent the previous year's flu vaccines were delivered in advance of the winter flu season.

Also presented are data regarding deaths from influenza and pneumonia for the period 2015-2024.

Two newly identified datasets on positive influenza test results for hospital admissions have recently been identified and are currently undergoing validation and quality assurance. As a result, all positive influenza test result data have been excluded from this publication to ensure that statistics presented in this report reflect the true level of positive test results for hospital patients. Subject to the completion of quality assurance processes, the validated data will be included in this report as soon as possible.

Summary

In 2024-2025:

- the number of flu-like illness GP consultations peaked around mid- January 2024, and was higher than in the 2023-2024 season
- the flu-like illness activity at the peak of the season was greater than the average across pre-pandemic seasons levels
- Jersey succeeded in delivering influenza (flu) vaccinations to nearly 31,000 individuals
- the proportion of patients aged 65 years and over who were vaccinated was 65%. This was lower than the World Health Organization (WHO) flu vaccine uptake ambition of 75%
- the proportion of pre-school children aged 2-4 years who were vaccinated in nurseries or at GP surgeries (53%) was lower than in the previous season (60%)
- vaccine uptake in the school-aged children programme (57%) was similar to the previous season (58%)
- over a quarter of individuals aged 50-64 years (29%) were vaccinated in 2024-2025; this was the same proportion as in the previous programme (29%)
- in 2024, there were around 25 deaths due to Influenza and Pneumonia, similar to the numbers seen in some previous years

Background

Flu is caused by the influenza virus, and for some groups of people can be serious (e.g., they can develop a serious complication such as pneumonia). However, there are other viruses (i.e., not the influenza virus) that can cause similar symptoms. These other 'flu-like' viruses are often mistaken for influenza and could be referred to as "the flu" but are in fact a different illness. The only way to confirm that a 'flu-like' illness is caused by the influenza virus is to conduct a laboratory test, which in many cases is not necessary.

It is important to note that interpretation of influenza surveillance data over the last few years should consider the impact of the COVID-19 pandemic; data reported from the various influenza surveillance systems was affected by changing social and behavioural factors during the pandemic, as well as in testing regimes. The data presented should only be used as a guide, especially when compared to earlier influenza seasons.

'Flu-like' illness

The number of resident patients presenting to their GP with 'flu-like' illness is taken from the General Practitioner Central Server (GPCS), EMIS. These people are not tested for influenza, so it is not known what proportion of them have actual influenza, and what proportion have a 'flu-like' illness.

Also note that not everyone with 'flu-like' illness will attend their GP. This metric is intended as an indicator of the relative amount of flu-like illness circulating in the community and is useful for comparing patterns year on year.

Numbers of 'flu-like' illnesses presenting to GPs

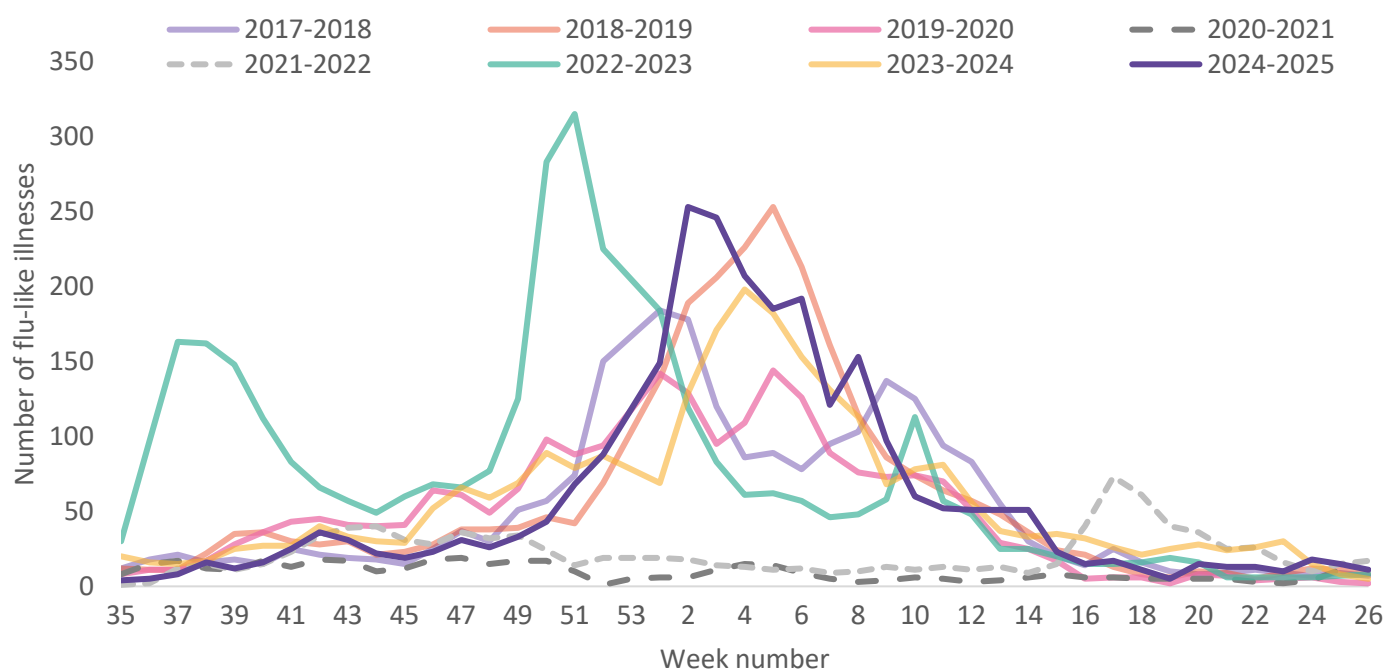
Figure 1 shows the numbers recorded per week as presenting to their GP with 'flu-like' symptoms over each flu season between the 2017-2018 winter and the 2024-2025 winter. The individual flu-season profiles vary, in terms of the peak number of cases seen, the timing of the increase and decrease in the number of cases, and shape of profile.

In 2024-2025:

- flu-like illness activity continued to be at levels seen prior to the COVID-19 pandemic, similar to the 2022-2023 and 2023-2024 winter seasons
- Jersey experienced one peak of influenza, similar to the 2023-2024 winter season; this differs from the previous 2022-23 season that saw increased activity in the autumn; this pattern was similarly observed across the UK¹
- the peak number of flu-like illness cases was earlier and higher than the previous 2023-2024 season, reaching the same level of activity as the pre-pandemic 2018-19 season (Figure 1)

¹ [Influenza in the UK, annual epidemiological report: winter 2024 to 2025 - GOV.UK](#)

Figure 1: ‘Flu-like’ illness profiles by week and season, Jersey (2017-2018 to 2024-2025)



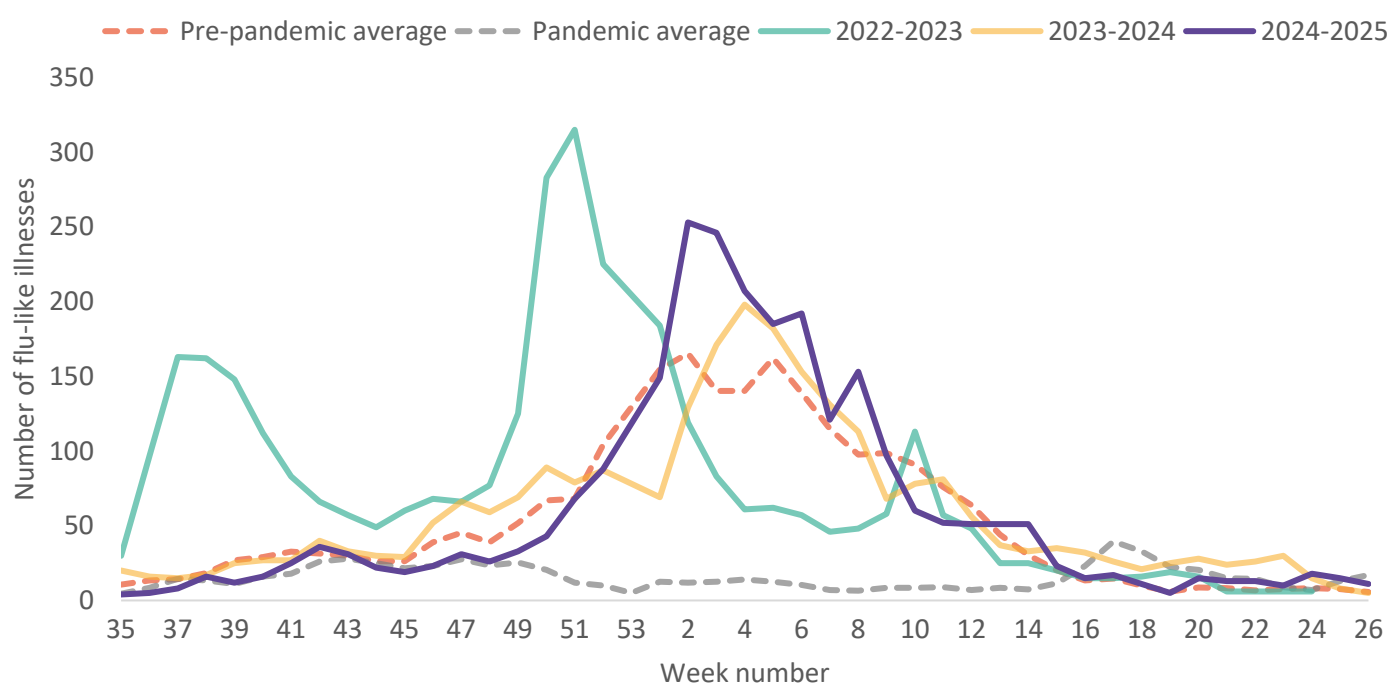
Source: General Practitioner Central Server (GPCS), EMIS

Data from individual flu seasons can be combined to create an ‘average’ profile (calculated as a mean) (see Figure 2). This average profile appears smoother, capturing the earliest rises and latest declines across the individual seasons, while flattening the peak number of cases.

Because flu activity was unusually low during the 2020-2021 and 2021-2022 winter seasons, the “pre-pandemic average” (based on the three seasons before COVID-19) is shown in Figure 2 to illustrate the typical seasonal pattern.

The most recent season (2024-2025) is shown in purple, alongside the 2022-2023 season in green and the 2023-2024 season in yellow. Comparing these profiles with the pre-pandemic average shows that flu-like illness activity in 2023-2024 and 2024-2025 was more in line with pre-pandemic levels than in 2022-2023.

Figure 2: ‘Flu-like’ illness profiles by week and season, Jersey: Pre-pandemic average (2017-2018, 2018-2019, 2019-2020), Pandemic average (2020-2021, 2021-2022), 2022-2023, 2023-2024, and 2024-2025



Source: General Practitioner Central Server (GPCS), EMIS

Influenza vaccination

Annual vaccination is the best protection against the flu. The most common strains of the virus that cause influenza change every year, and the vaccine also changes every year to match these strains. A seasonal flu vaccine plan is co-ordinated every year in Jersey to prevent flu amongst those who are at a higher risk of flu-associated illness and mortality.

Flu vaccination programme

The programme provides direct protection to those at higher risk of flu associated morbidity and mortality, including older people, pregnant women, and those in clinical risk groups. The Jersey programme is guided by advice from the Joint Committee on Vaccination and Immunisation (JCVI).²

During the 2024-2025 flu seasons, an expanded offer was continued which enabled all those aged 50 to 64 years to receive the flu vaccine as part of the funded programme (flu vaccine offered free of charge).

The below groups were eligible for a flu vaccine from October 2024:

- infants 6 months to 2 years of age in a clinical risk group³
- pre-school children aged 2, 3 and 4 years
- school-aged children reception up to year 11
- at-risk 16 to 49-year-olds
- people aged 50 to 64, and those over 65
- pregnant women
- residents in long-stay care homes
- households of those on the shielded patient list or of immunocompromised individuals
- home carers
- care home and domiciliary staff
- frontline health and community services staff

Additionally, there was an offer for the flu and COVID-19 autumn booster vaccines to be administered together, to those who were eligible. Mobile vaccination units were also used for visits to care homes and private houses to improve the vaccination rate for older Islanders.

- during the Winter Vaccination Programme, Jersey succeeded in delivering influenza (flu) vaccinations to nearly 31,000 individuals

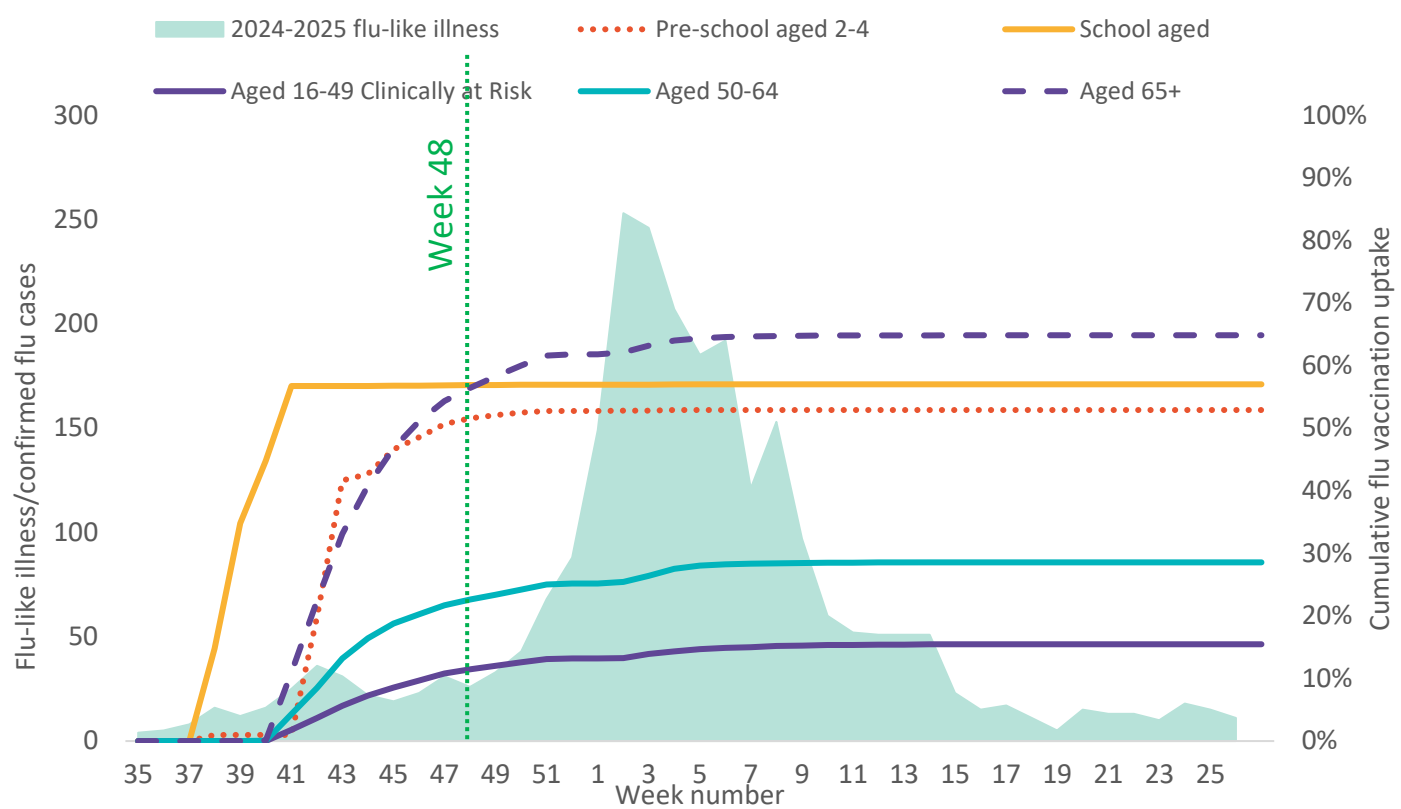
Key to the vaccination programme being successful is having a high proportion of the targeted populations vaccinated before influenza starts circulating. The vaccine takes approximately **two weeks** to become fully effective after being administered. Therefore, to achieve the best protection for the at-risk population, most of those eligible for flu vaccination should be vaccinated at least two weeks prior to the expected onset of influenza (i.e. by week 48 as onset is typically December onwards, Figure 1).

Figure 5 shows the flu vaccination profiles over the 2024-2025 season compared to the 'flu-like' illness and confirmed influenza profiles. The majority of flu vaccinations had been delivered by week 48 (Table 1); for example, of those aged 65+ vaccinated, 87% had received their vaccination by week 48, ahead of the onset of peak flu-like illness circulation.

² [Joint Committee on Vaccination and Immunisation - GOV.UK \(www.gov.uk\)](https://www.gov.uk/government/organisations/joint-committee-on-vaccination-and-immunisation)

³ See Notes section for further detail on clinical risk groups

Figure 5: Cumulative flu vaccination uptake of target groups and flu-like illness profile, Jersey (2024-2025)



Source: General Practitioner Central Server (GPCS), EMIS; Community Pharmacy Server, PharmOutcomes; Child Health Immunisation System (CHIS);

Table 1: Percentage flu vaccination uptake of each eligible age group by the end of the flu season, by week 48, and percentage of the total number of those vaccinated that were vaccinated by week 48 (i.e. % of vaccinations that were administered at least two weeks before the expected arrival of flu season) (2024-2025)

Age Group	% of group vaccinated by end of flu season	% of group vaccinated by week 48	Of those vaccinated by the end of flu season, % vaccinated by week 48
children aged 2, 3 and 4 years	53	52	97
School aged children aged 4 to 16 years	57	57	100
people aged 16-49-years-old	15	11	74
people aged 50-64-year-old	29	23	79
people aged 65 and over	65	56	87

Source: General Practitioner Central Server (GPCS), EMIS; Community Pharmacy Server, PharmOutcomes; Child Health Immunisation System (CHIS)

The ambition for flu vaccine coverage is to reach or exceed 75% uptake for people aged 65 years and over, as recommended by the World Health Organization (WHO).

Although all countries of the UK use standardised specifications to extract uptake data from IT information systems in primary care, there are some differences in extraction specifications, so comparisons between Jersey and the four constituent countries of the United Kingdom should be made cautiously.

Flu vaccine for pre-school children aged 2, 3 and 4 years⁴

Pre-school children are offered vaccination to help protect them and to reduce flu transmission to other vulnerable groups in the community. The vaccine programme for 2024-2025 was the eighth year of the nursery-based immunisation programme, where GPs and Practice Nurses went into nurseries to offer the nasal flu vaccine. The vaccine was also available to children of this age-group through GP surgeries.

- the number of vaccines given to pre-school aged children in 2024-2025 (920) was lower than the previous season (1,040)⁵
- the flu vaccine uptake among children aged 2-4 in the 2024-2025 season (53%) has decreased compared to 2023-2024 (60%), but is similar to that of 2022-2023 (52%)
- in England, 43% of children aged 2-3 years received a flu vaccination during the 2024-2025 winter season⁶

Table 2. Percentage flu vaccination uptake of pre-school children aged 2-4 years old, by vaccination setting, Jersey (2018-2019 to 2024-2025)

	2018-19 winter %	2019-20 winter %	2020-21 winter %	2021-22 winter %	2022-23 winter %	2023-24 winter %	2024-25 winter %
% of children aged 2-4 years vaccinated in nursery	39	47	55	48	43	51	42
% of children aged 2-4 years vaccinated in GP surgery	18	17	12	10	9	9	11
% of children aged 2-4 years vaccinated in other healthcare setting	-	-	3	-	-	-	-
Total % uptake for children aged 2-4 years	58	64	69	57	52	60	53

Source: General Practitioner Central Server (GPCS), EMIS; Child Health Immunisation System (CHIS)

Flu vaccine for compulsory school-aged children (Reception to Year 11)

The children's nasal flu vaccination programme was introduced in 2014-2015 with children in Reception classes (aged 4 to 5 years) being offered the vaccine via a school-based programme. Since 2018-2019 the nasal flu vaccine has been offered to all compulsory school-aged children (Reception to Year 11 inclusive). A breakdown of the children immunised at school against flu, by year group, is given in Table 3.

Around 6,860 school-aged children had the flu vaccine in the 2024-2025 winter:

- 6,828 influenza vaccines were given in primary and secondary schools, whilst 32 were given at GP surgeries
- the number of vaccines given to school-aged children in 2024-2025 (6,860) was lower than the previous season (7,098)
- flu vaccine uptake for school-aged children in 2024-2025 (57%) was similar to 2022-2023 (58%)
- in England, 50% of all school aged children (4-16 years) were vaccinated in 2024-2025⁷

⁴ Pre-school children (2-, 3- and 4-year-olds) whose date of birth was between 01/09/2020 and 31/08/2020.

⁵ Numbers rounded to nearest 10.

⁶ Seasonal influenza vaccine uptake in GP patients in England: winter season 2024 to 2025 - GOV.UK

⁷ Seasonal influenza vaccine uptake in children of school age in England: winter season 2024 to 2025 - GOV.UK

Table 3. Percentage flu vaccination uptake of school-aged children in school, by year group, Jersey (2018-2019 to 2024-2025)⁸

	2018-19 winter %	2019-20 winter %	2020-21 winter %	2021-22 winter %	2022-23 winter %	2023-24 winter %	2024-25 winter %
Reception	66	66	78	76	69	65	66
Year 1	63	66	73	75	63	68	62
Year 2	60	64	77	69	73	68	67
Year 3	61	62	71	73	60	64	65
Year 4	62	62	71	69	64	60	64
Year 5	57	61	70	71	55	63	61
Year 6	57	57	72	69	66	59	63
Year 7	55	54	63	62	58	51	59
Year 8	51	53	62	57	56	48	50
Year 9	48	50	58	59	48	52	55
Year 10	52	49	63	59	57	47	56
Year 11	57	49	56	54	48	47	40

Source: General Practitioner Central Server (GPCS), EMIS; Child Health Immunisation System (CHIS)

Flu vaccine for adults

- around 812 influenza vaccines were given to individuals aged 16-49 years who identified that their primary reason for a flu vaccination being because they had certain chronic health conditions (e.g., a long-term heart condition, diabetes) or were immunosuppressed⁹
- around 230 influenza vaccines were given to pregnant women¹⁰ from 7 October 2024 to 31 March 2025. This was a slight increase from the number given in the 2023-24 winter, but is lower number than all winter seasons prior to 2022-23 (see Table 4)

⁸ Table does not include the children immunised at GP practices, or home-schooled children.

⁹ Individuals eligible under multiple criteria may have their eligibility recorded under a different category at the point of vaccination. This may lead to an undercount of vaccinations attributed to this eligibility group

¹⁰ Pregnant women are not obliged to disclose their pregnancy at time of vaccination

Table 4. Number of patients in the pregnant women category who received a flu vaccination, Jersey (2018-2019 to 2024-2025)

	2018-19 winter	2019-20 winter	2020-21 winter	2021-22 winter	2022-23 winter	2023-24 winter	2024-25 winter
Pregnant women	500	510	550	340	240	190	230

Source: General Practitioner Central Server (GPCS), EMIS; Community Pharmacy Server, PharmOutcomes

In 2024-2025:

- around 65% of people aged 65 and over received a seasonal flu vaccination in Jersey. This was lower than the uptake for this age group
- uptake in Jersey among those aged 50-64 years old was 29%, the same as the 2023-2024 season; this is significantly lower than it was in previous seasons (Table 5). Note that in England, those aged 50-64 years old and not in a clinical risk group were not eligible for the flu vaccine¹¹
- the flu vaccine uptake among those aged 65 and over in Jersey was lower than across the devolved nations: England saw a vaccine uptake of 78%¹¹ in those aged 65 and over, Scotland (90%),¹² Northern Ireland (78%)¹³ and Wales (73%)¹⁴

Table 5. Percentage flu vaccination uptake of those aged 50-64 years and aged 65 and over, Jersey (2018-2019 to 2024-2025)

	2018-19 winter %	2019-20 winter %	2020-21 winter %	2021-22 winter %	2022-23 winter %	2023-24 winter %	2024-25 winter %
Adults aged 50-64 years	-	-	45	51	47	29	29
Adults aged 65 and over	60	55	78	84	80	74	65

Source: General Practitioner Central Server (GPCS), EMIS; Community Pharmacy Server, PharmOutcomes

Influenza and pneumonia deaths

It is possible to provide an estimate of deaths due to influenza by reporting on the underlying cause of death (see Notes section for details). The underlying cause of death is defined by the WHO as "the disease or injury which initiated the train of morbid events leading directly to death, or the circumstances of the accident or violence which produced the fatal injury".¹⁵

The number of deaths for Jersey with an underlying cause of Influenza and Pneumonia (ICD-10 code J09-J18) which is used as a proxy for flu deaths, for the most recent calendar years (2015-2024) where data is available are shown in Table 6.

In 2024, there were around 25 deaths due to Influenza and Pneumonia, similar to the numbers seen in previous years.

¹¹ [Seasonal influenza vaccine uptake in GP patients in England: winter season 2024 to 2025 - GOV.UK](#)

¹² [NHS Scotland - Publications](#)

¹³ [Seasonal Influenza Vaccination Surveillance in Northern Ireland 2024-25 | HSC Public Health Agency](#)

¹⁴ [Annual influenza surveillance and influenza vaccination uptake reports - Public Health Wales](#)

¹⁵ [WHO Medical Certification Cause of Death](#)

Table 6: Annual Influenza and Pneumonia deaths (ICD-10 code J09-J18, 2015-2024)

Year	Influenza and Pneumonia Deaths*
2015	30
2016	30
2017	30
2018	20
2019	20
2020	15
2021	25
2022	30
2023	30
2024	25

Source: Public Health mortality database

*Counts are rounded to the nearest multiple of 5

Please note that it still may not be possible to know exactly how many people die from seasonal flu each year, as Influenza may not always be listed on death certificates of people who die from flu-related complications. There are several reasons for this:

- serious complications can be triggered by flu; flu can make chronic medical problems worse
- many flu-related deaths can also occur one or two weeks after a person's initial infection, either because the person may develop a secondary bacterial co-infection or because influenza can aggravate an existing chronic illness
- most people who die from flu-related complications are not tested for flu or may not have sought medical care until later in their illness when influenza can no longer be detected from respiratory samples

Notes

Data Sources

Population estimates

Where Figures are expressed as a proportion of the population, most recent population estimates¹⁶ produced by Statistics Jersey have been used as denominators. Note that there may be some differences in prevalence estimates compared to previous versions of this report, and older reports used interim estimates in lieu of the official Figures being published by Statistics Jersey.

Vaccination data

The vaccination data for this report are derived from GP Central Server (EMIS), Community Pharmacy server (PharmOutcomes) and records kept by those administering the school and nursery programmes.

Flu vaccination uptake in each of the eligible groups were calculated using denominators (total in group) from the following sources:

- number of pre-school aged children, and compulsory school aged children: Child Health Immunisation System (CHIS), CarePlus
- number of adults aged 16 and over: population estimates produced by Statistics Jersey

Mortality data

The Marriage and Civil Status (Jersey) Law 2001 requires all deaths to be registered with the Superintendent Registrar within 5 days of the date of death unless they have been referred to the Viscount.

Underlying cause of death is classified using the International Statistical Classification of Diseases, Injuries and Causes of Death (tenth revision, ICD-10). Coding of cause of death of Jersey registered deaths is undertaken by the Office for National Statistics on a quarterly basis. The Office for National Statistics (ONS) determines the leading causes of death using a detailed list based on one developed by the World Health Organization (WHO). This list uses more specific groupings than the broad group level, splitting causes such as cancer and circulatory diseases into different subtypes, with the aim to provide policymakers with enough detail to generate appropriate health policies and interventions. To identify the leading causes of death for this analysis, the ONS used a grouping originally produced by the World Health Organization (WHO).

Deaths caused by influenza and pneumonia are identified in the ICD-10 revision by the following: Influenza is coded J09-J11; Pneumonia is coded J12-J18

Data quality and completeness

The data quality and completeness of data extracted from the GP central server cannot be assured, however where variation between GP practices is identified, this is fed back to individual surgeries for further checks. Figures pulled are also compared to previous year figures to see where large changes have occurred, these can then be further investigated.

There are limitations to the data reported for vaccinations of pregnant women in this report. The completeness of the data depends on the recording of pregnancy as the main reason for vaccination, which may not necessarily happen for all pregnant women receiving a vaccine.

This report has important limitations related to vaccine data for individuals at clinical risk (see list below). The completeness of the data depends on the recording of the risk situation that vaccinators consider as the main reason for vaccination, for example, some of those who may be eligible due to certain health conditions may also be eligible

¹⁶ [Annual Population Estimates by Age and Sex – Statistics Jersey, October 2024](#)

as they are an employee (e.g., health worker) or if aged between 50 and 64 years of age. Therefore, coverage of the clinically at-risk group is an estimate only.

Clinical risk groups eligible for flu vaccination

The following provides a summary of the clinical risk groups eligible for flu vaccination in 2024-2025 among those aged 16-49:

- Chronic respiratory disease such as asthma requiring regular inhaled steroids, or chronic obstructive pulmonary disease (COPD)
- Chronic heart disease
- Chronic kidney disease at stage 3, 4 or 5
- Chronic liver disease
- Chronic neurological disease such as Parkinson's disease, motor neurone disease
- Learning disability
- Diabetes
- Immunosuppression due to disease such as HIV/AIDS or treatment such as cancer treatment (and household contacts of at-risk individuals)
- Asplenia or dysfunction of the spleen
- Morbidly obese (class III obesity). This is defined as those with a Body Mass Index (BMI) of 40 or above, aged 16 or over.